UnitedHealthcare®

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January 20, 2016

Matthew Kempton
CEO/Administrator
Stephens Memorial Hospital
200 S. Geneva Street
Breckenridge, TX 76242-4702

Dear Mr. Kempton:

Please find enclosed one (1) fully executed original Assignment and Assumption Agreement between UnitedHealthcare of Texas and Stephens Memorial Hospital.

If you have any questions, please do not hesitate to call me at (952) 202-4146. We look forward to a continued strong working relationship with Stephens Memorial Hospital!
Sincerely,

Pamela Carico

Pam Carico
Sr. Network Account Manager

Enclosure

Assignment and Assumption Agreement

This Assignment and Assumption Agreement (this "Agreement") is entered into by and between United HealthCare Insurance Company, contracting on behalf of itself, United HealthCare of Texas, Inc., PacifiCare of Texas, Inc., Evercare of Texas, LLC, and the other entities that are its affiliates (collectively referred to as "United"), and Stephens Memorial Hospital ("Facility") and Stephens Memorial Hospital District.

This Agreement is effective on July 1, 2015 (the "Effective Date").

Recitals

WHEREAS, United and Facility entered into a Facility Participation Agreement effective September 10, 2008 (the "Facility Agreement");

WHEREAS, Facility has entered into a transaction pursuant to which all of the assets and liability of Stephens Memorial Hospital ("Facility")were transferred to Stephens Memorial Hospital District (the "Transaction");

WHEREAS, Facility desires to assign the Facility Agreement and Stephens Memorial Hospital District desires to assume Facility's rights and obligations under the Facility Agreement;

WHEREAS, Facility may not assign the Facility Agreement without United's written consent thereto.

NOW, THEREFORE, in consideration of the promises and the mutual covenants herein stated, and other good and valuable consideration, the parties hereto agree as follows:

1. Recitals. The parties hereto acknowledge and agree that the above stated recitals are true and correct. Further, the recitals are incorporated herein by this reference.

2. Facility Agreement. The Facility Agreement is attached to this Agreement.

3. Assumption. Stephens Memorial Hospital District shall assume all rights and obligations under the Facility Agreement which may arise on or after the Effective Date. For purposes of this Agreement, rights and obligations relating to payment of a claim, or additional payment of an underpayment or refund of an overpayment of a given claim, arise on the date of service.

4. Assignment. Facility assigns to Stephens Memorial Hospital District all rights and obligations under the Facility Agreement which may arise on or after the Effective Date.

5. Consent. As described herein, United consents to the assignment of the Facility Agreement by Facility to Stephens Memorial Hospital District and the assumption of the Facility Agreement by Stephens Memorial Hospital District.

6. Effect on the Facility Agreement. The terms and conditions of the Facility Agreement remain in full force and effect.

IN WITNESS WHEREOF, the parties hereto have executed this Agreement as of the Effective Date written above.

United HealthCare Insurance Company, contracting on behalf of itself, United HealthCare of Texas, Inc. and the other entities that are its affiliates	Stephens Memorial Hospital District
By:	By:
Name: Michelle Hartmann	Name: Matthew B. Keyston
Title: VP, Network Mgt	Title: CEO
Date: 12.15.15	Date: 12/15/15

All Payer Appendix

Stephens Memorial Hospital

Chargemaster Adjustment Effective 04/01/2016

APPLICABILITY

Unless another appendix to this Agreement applies specifically to a particular Benefit Plan as it covers a particular Customer, the provisions of this Appendix apply to Covered Services rendered to Customers covered by Benefit Plans sponsored, issued or administered by all Payers.

SECTION 1
Definitions

Unless otherwise defined in this Section 1, capitalized terms used in this Appendix shall have the meanings assigned to them in this Agreement.

Admission: The admittance of a Customer to a licensed hospital bed, excluding Observation. Admission applies only to those services provided by order of a Physician.

Covered Service: A health care service or product for which a Customer is entitled to receive coverage from a Payer, pursuant to the terms of the Customer's Benefit Plan with that Payer.

Customary Charge: The fee for health care services charged by Facility that does not exceed the fee Facility would ordinarily charge another person regardless of whether the person is a Customer.

Customer Expenses: Copayments, deductibles, or coinsurance that are the financial responsibility of the Customer according to the Customer's Benefit Plan.

Eligible Charges: The Customary Charge for Covered Services, except for Covered Services listed under sections 3.4 and 3.6 of this Appendix.

Institutional Claim: Any UB-04 or electronic version or successor form.

MS-DRG (Medicare Severity Diagnosis-Related Groups): A system of classification for inpatient hospital services based on the principal diagnosis, secondary diagnoses, surgical procedures, age, sex, and discharge status.

Observation: Services furnished by a Facility on the Facility's premises, regardless of the length of stay, including use of a bed and periodic monitoring by a Facility's nursing or other staff, which are reasonable and necessary to evaluate an outpatient condition or determine the need for a possible Admission to the Facility as an inpatient. Observation applies only to those services provided by order of a Physician.

Outpatient Encounter: Covered Services rendered to a Customer by Facility on an outpatient basis from the time of registration for outpatient services until discharge.

Payment Method: A methodology for determining contract rates under this Appendix.

Per Case: The Payment Method designated "Per Case" in this Appendix and applicable to Covered Services rendered to a Customer during an entire Admission or one Outpatient Encounter. Unless otherwise specified in this Appendix, payment under the Per Case Payment Method, less any applicable Customer Expenses, shall be considered payment in full for all Covered Services rendered to the Customer including, but not limited to, physician and other professional fees billed by Facility on an Institutional Claim, "pre-admission diagnostic services" (as defined by CMS) that occurs within three calendar days prior to Admission, nursing care, Observation, critical care,

surgical services (including services rendered by non-Physician personnel, regardless of whether those personnel are employed by Facility and regardless of whether those services are characterized as professional services), diagnostic and therapeutic services (including but not limited to diagnostic imaging), ancillary services, durable medical equipment, supplies (including, but not limited to, anesthesia supplies), medications, and room and board.

Per Diem: The Payment Method designated "Per Diem" in this Appendix and applicable to Covered Services rendered to a Customer for each day of an Admission of a Customer. Unless otherwise specified in this Appendix, payment under the Per Diem Payment Method, less any applicable Customer Expenses, shall be considered payment in full for all Covered Services rendered to the Customer during each day of the Admission including, but not limited to, physician and other professional fees billed by Facility on an Institutional Claim, nursing care, Observation, critical care, surgical services (including services rendered by non-Physician personnel, regardless of whether those personnel are employed by Facility and regardless of whether those services are characterized as professional services), diagnostic and therapeutic services (including, but not limited, to diagnostic imaging), ancillary services, durable medical equipment, supplies (including, but not limited to, anesthesia supplies), medications, and room and board.

Per Unit : The flat rate Payment Method designated "Per Unit" in this Appendix and applicable to Covered Services rendered to a Customer for each unit of service performed within an Outpatient Service Category for which a Per Unit Payment Method is indicated in the Outpatient Service Categories Table of this Appendix. Unless otherwise specified in this Appendix, payment under the Per Unit Payment Method, less any applicable Customer Expenses, shall be considered payment in full for all Covered Services rendered to the Customer including, but not limited to, physician and other professional fees billed by the Facility on an Institutional Claim (including services rendered by non-Physician personnel, regardless of whether those personnel are employed by Facility and regardless of whether those services are characterized as professional services), nursing care, diagnostic and therapeutic services, durable medical equipment, supplies (including, but not limited anesthesia supplies), medications, facility and ancillary services, and, if applicable room and board. Units reported for Covered Services for which the contract rate is a Per Unit, must always equal the number of times a procedure or service is performed.

Per Unit via Fee Schedule: The Payment Method designated "Per Unit via Fee Schedule" in this Appendix, based on the CPT/HCPCS specific fee listed in the applicable fee schedule for each unit of service and applicable to Covered Services rendered to a Customer for services performed within an Outpatient Service Category for which a Per Unit via Fee Schedule Payment Method is indicated in the Outpatient Service Categories table of this Appendix. Unless otherwise specified in this Appendix, payment under the Per Unit via Fee Schedule Payment Method, less any applicable Customer Expenses, shall be considered payment in full for all Covered Services rendered to the Customer including, but not limited to, physician and other professional fees billed by the Facility on an Institutional Claim (including services rendered by non-Physician personnel, regardless of whether those personnel are employed by Facility and regardless of whether those services are characterized as professional services), nursing care, diagnostic and therapeutic services, durable medical equipment, supplies (including, but not limited anesthesia supplies), medications, facility and ancillary services, and, if applicable room and board. Units reported for Covered Services for which the contract rate is a Per Unit via Fee Schedule, must always equal the number of times a procedure or service is performed.

Per Visit : The flat rate Payment Method designated "Per Visit" in this Appendix and applicable to Covered Services rendered to a Customer on one-calendar day period, for each Outpatient Service Category billed for which a Per Visit Payment Method is indicated in the Outpatient Service Categories Table of this Appendix. Unless otherwise specified in this Appendix, payment under the Per Visit Payment Method, less any applicable Customer Expenses, shall be considered payment in full for all Covered Services rendered to the Customer including, but not limited to, physician and other professional fees billed by the Facility on an Institutional Claim (including services rendered by non-Physician personnel, regardless of whether those personnel are employed by Facility and regardless of whether those services are characterized as professional services), nursing care, diagnostic and therapeutic services, durable medical equipment, supplies (including, but not limited to anesthesia supplies), medications, facility and ancillary services, and, if applicable, room and board. Facility is required to identify each date of service when submitting claims spanning multiple dates of service.

Percentage Payment Rate: The percentage applied to Facility's detail line item Eligible Charge to determine the contract rate for those Covered Services for which the contract rate is calculated as a percentage of Eligible Charges. This shall be referred to as "PPR" in this Appendix.

Percentage Payment Rate Per Case: The percentage applied to Facility's total Eligible Charge of the claim to determine the contract rate for those Covered Services for which the contract rate is calculated as a percentage of Eligible Charges. Unless otherwise specified in this Appendix, payment under the Percentage Payment Rate Per Case Payment Method, less any applicable Customer Expenses, shall be considered payment in full for all Covered Services rendered to Customer including, but not limited to, physician and other professional fees billed by Facility on an Institutional Claim, "pre-admission diagnostic services" (as defined by CMS) that occurs within three calendar days prior to Admission, nursing care, Observation, critical care, surgical services (including services rendered by non-Physician personnel, regardless of whether those personnel are employed by Facility and regardless of whether those services are characterized as professional services), diagnostic and therapeutic services (including but not limited to diagnostic imaging), ancillary services, durable medical equipment, supplies (including, but not limited to, anesthesia supplies), medications, and room and board. This shall be referred to as "PPR Per Case" in this Appendix.

Physician: A Doctor of Medicine ("M.D.") or a Doctor of Osteopathy ("D.O.") or another health care professional as authorized under state law and Facility bylaws to admit or refer patients for Covered Services.

SECTION 2
Contract Rate for Covered Services

2.1 Contract Rate. For Covered Services rendered by Facility to a Customer, the contract rate will be the lesser of (1) Facility's aggregate Eligible Charges, or (2) the aggregate applicable contract rate determined in accordance with Sections 2.2, 2.3, 3 and/or 4 of this Appendix. Payment by Payer of the contract rate under this Appendix will be less any applicable Customer Expenses, and is subject to the requirements set forth in the Agreement regarding timely submission of a complete claim and compliance with applicable Protocols such as notification of Admission.

2.2 Inpatient Covered Services. For the provision of Covered Services to a Customer during an Admission, the contract rate is determined as described in this Section 2.2. The contract rate for an Admission is the contract rate in effect on the date the Admission begins.

Table 1A: Inpatient Service Category Table

SERVICE CATEGORY	PAYMENT METHOD	CONTRACT RATE
Medical Refer to CMS list of MS-DRGs However, this service category does not apply where one of these services is billed in connection with Covered Services included in any of the other service categories on this Inpatient Service Category Table or Section 3.6 _____	**Per Diem**	$ 1,750
Surgical Refer to CMS list of MS-DRGs However, this service category does not apply where one of these services is billed in connection with Covered Services included in any of the other service categories on this Inpatient Service Category Table or Section 3.6 _____	**Per Diem**	$ 2,000
ICU/CCU/PICU Includes the following Revenue Codes. Revenue Codes: 0200-0203, 0207-0212, 0219	**Per Diem**	$ 1,750
Hospice Revenue Codes 0115, 0125, 0135, 0145, 0155, 0655-0656	**Per Diem**	$ 600

Nursery		
• Normal Newborn: MS-DRG 795	Per Diem	$ 250
• Lower Level Neonate: MS-DRGs 789, 792, 794	Per Diem	$ 300
• Higher Level Neonate: MS-DRGs 791, 793	Per Diem	$ 350
• Severe Level Neonate : MS-DRG 790	Per Diem	$ 400

Obstetrics (Mother only)*		
Vaginal delivery MS-DRGs: 767-768, 774-775	Per Case	$ 1,750
Cesarean Section MS-DRGs: 765-766	Per Case	$ 3,000

False Labor MS-DRG: 780	Per Diem	$ 500

Rehabilitation~ MS-DRG: 945-946 or Revenue Codes 0118, 0128, 0138, 0148, 0158	Per Diem	$ 850

Hospital Sub-Acute Revenue Codes 0190-0199	Per Diem	$ 750

Inpatient Skilled Nursing Services~ (see note ~ below) Bill Types 211-219	Per Diem	$ 750

Notes to Table 1A

*Covered Services rendered to a mother and her newborn child shall be paid as separate Admissions.

~ If Facility has a separate Inpatient Skilled Nursing unit, Hospice unit, or Rehabilitation unit, the charges for the Inpatient Skilled Nursing, Hospice, or Rehabilitation stay are to be submitted separately from the acute hospital stay.

Additional information regarding MS-DRGs under this Appendix

The following applies to MS-DRGs as used in this Appendix:

-The contract rate for a new, replacement, or modified MS-DRG code(s) will be at the existing contract rate for the appropriate MS-DRG(s) it replaced or modified.

-All changes in the definition of MS-DRG's specified in the Final Rule shall be implemented under this Appendix on or before January 1, following publication in the Federal Register. Until changes in the definition are implemented under this Appendix, the previous definitions will apply. Claims with discharge dates 10/1 and later, that are processed during the period in between the CMS effective date and United's implementation date will continue to have the previous MS-DRG grouper applied. Claims with discharge dates 10/1 and later that are processed following United's implementation date for the MS-DRG grouper updates will have the new grouper applied.

2.3 Outpatient Covered Service Categories

2.3.1 Observation, Outpatient Therapeutic, Diagnostic, Emergency, Urgent Care Covered Services.

For the provision of Observation, therapeutic, diagnostic, Emergency, and Urgent Care Covered Services rendered by Facility to a Customer on an outpatient basis (except for Outpatient Procedures addressed in section 2.3.2 of this Appendix), the contract rate shall be determined according to this Section 2.3.1. For any Outpatient Covered Service, appropriate CPT and/or HCPCS codes must be submitted on the Institutional Claim in order to be eligible for reimbursement.

If more than one type of Covered Services for which a Per Visit Payment Method applies is provided during one calendar day, Facility shall be paid the applicable Per Visit Payment Method for each Covered Service; provided, however, if the Customer receives any Covered Service for which a Per Case applies, all Covered Services which would otherwise be paid pursuant to a Per Visit Payment Method shall be included in the Per Case contract rate and will not be separately reimbursed.

If multiple Covered Services found on the Outpatient Service Category table are provided during one calendar day, and provided that at least one of the Covered Services have a contract rate based on a Per Case or Per Visit Payment Method, then the Revenue Codes not specifically listed on the Outpatient Service Category table, will not be separately reimbursed; instead, such services will be considered to have been included within contract rate for the Covered Service listed on the Outpatient Service Category table with a Per Case or Per Visit Payment Method.

For example, Service A would be listed on the Outpatient Service Category table with a Payment Method of Per Visit. Service B would be listed on the Outpatient Services Category table with a Payment Method of Percentage Payment Rate. The Supplies revenue code would not be listed on the Outpatient Services Category table and therefore would not be separately reimbursed under the Percentage Payment Rate specified in the Appendix. Specifically, Service B has a contract rate under a Percentage Payment Rate Payment Method for revenue code YYY only; therefore, no separate reimbursement is made for revenue code 0270 since this is considered included within the contract rate for Service A within the Per Visit::

Revenue Code	Customary Charge	Payment Method
XXX - Service A	$500	Per Visit
YYY - Service B	$100	Percentage Payment Rate
0270 - Supplies	$ 50	No Separate Reimbursement

The contract rate for outpatient diagnostic and therapeutic Covered Services rendered by Facility to a Customer, for which a Per Case, Per Visit, Per Unit or Percentage Payment Rate Per Case Payment Method is detailed on Table 2 below, will be determined according to the table.

Table 2: Outpatient Diagnostic and Therapeutic Services Category Table

OUTPATIENT DIAGNOSTIC AND THERAPEUTIC SERVICES	PAYMENT METHOD	CONTRACT RATE
Gamma Knife (Revenue Codes 0333 with CPT Code 61793, 77371)	**Per Case**	$ 4,000
Rule Out Myocardial Infarction (Revenue Code 0762 with Principal ICD-9-CM Diagnosis Codes: 411.0-411.89, 413.0-413.9, 414.00-414.05, 786.50-786.59, V71.7) **(1)**	**Per Case**	$ 850
False Labor Revenue Codes: 0720-0722, 0724, 0729 with Principal ICD-9-CM Diagnosis Codes 644.10-644.13 **(1)**	**Per Case**	$ 500
Observation (Revenue Code: 0762) **(1)**	**Per Case**	$ 650

Emergency (Revenue Codes 0450-0452, 0459 with CPT 99281, 99282, 99283, 99284, 99285, 99291, G0380, G0381, G0382, G0383 or G0384) (1)		
Level 1: CPT/HCPC Codes: 99281, G0380	**Per Case By Level**	$ 325
Level 2: CPT/HCPC Codes: 99282, G0381	**Per Case By Level**	$ 575
Level 3: CPT/HCPC Codes: 99283, G0382	**Per Case By Level**	$ 700
Level 4: CPT/HCPC Codes: 99284, G0383	**PPR per Case to a Cap**	74.8% $1,600
Level 5: CPT/HCPC Codes: 99285, G0384	**PPR per Case to a Cap**	74.8% $3,000
Critical Care: CPT Code: 99291	**PPR per Case to a Cap**	74.8% $3,000
Note: In the event Facility does not specify one of the CPT codes listed above, the contract rate will default to the Level 1 rate above.		

Urgent Care (Revenue Code: 0456) (1)	**Per Case**	$ 100
IV Therapy (Revenue Codes 0260, 0269)	**PPR**	74.8 %
Oncology Treatment Revenue Codes: 0280, 0289	**PPR**	74.8%
Laboratory (Revenue Codes: 0300-0307, 0309, 0923, 0925)	**Per Unit**	$ 35
Pathology (Revenue Codes: 0310-0312, 0314, 0319)	**Per Unit**	$ 30
Other Diagnostic Radiology (Revenue Codes: 0320-0324, 0329)	**PPR**	74.8%
Cyber Knife – Initial Visit (Revenue Code 0333 with CPT Code G0339)	**Per Visit**	$ 4,500
Cyber Knife – Subsequent Visit (Revenue Code 0333 with CPT Code G0340)	**Per Visit**	$ 3,000
Radiation Therapy (Revenue Code: 0330, 0333, 0339) without CPT Code s 61793, 77371, G0339-G0340)	**PPR**	74.8%
Chemotherapy Administration (Revenue Codes: 0331-0332, 0335)	**PPR**	74.8%
Nuclear Medicine (Revenue Code: 0340-0342, 0349)	**PPR**	74.8%
Computerized Tomography (CT) Scan (Revenue Code: 0350-0352, 0359)	**Per Unit**	$ 750

Imaging Services (Revenue Codes: 0400, 0409)	**PPR**	74.8%
Diagnostic and Screening Mammography (Revenue Codes: 0401, 0403)	**PPR**	74.8%
Ultrasound Imaging (Revenue Code: 0402)	**PPR**	74.8%
Positron Emission Tomography (Revenue Code: 0404)	**Per Unit**	$ 600
Respiratory Services (Revenue Codes: 0410, 0412, 0419)	**PPR**	74.8%
Hyperbaric (Revenue Code: 0413)	**PPR**	74.8%
Physical Therapy (Revenue Codes: 0420-0424, 0429)	**PPR**	74.8%
Occupational Therapy (Revenue Codes: 0430-0434, 0439)	**PPR**	74.8%
Speech Therapy (Revenue Code: 0440-0444, 0449)	**PPR**	74.8%
Pulmonary Function (Revenue Code: 0460, 0469)	**PPR**	74.8%
Audiology (Revenue Codes: 0470-0472, 0479)	**PPR**	74.8%
Cardiology (Revenue Code: 0480, 0489)	**PPR**	74.8%
Cardiac Stress Test (Revenue Code: 0482)	**PPR**	74.8%
Echocardiology (Revenue Code: 0483)	**PPR**	74.8%
Ambulance - Land (Revenue Codes: 0540, 0542-0543, 0546-0549)	**PPR**	74.8 %
Ambulance - Air (Revenue Code: 0545)	**PPR**	74.80 %
Magnetic Resonance Imaging (Revenue Code: 0610-0612, 0614-0616, 0618, 0619)	**Per Unit**	$ 900
Labor Room/Delivery Services (Revenue Code: 0720-0722, 0724, 0729 without Principal Diagnosis Codes 644.10-644.13)	**PPR**	74.8%
EKG and ECG (Revenue Codes: 0730, 0739)	**PPR**	74.8%
Holter Monitor/Telemetry (Revenue Codes: 0731-0732)	**PPR**	74.8%

EEG (Revenue codes 0740 without CPT codes 95805-95811)	**PPR**	74.8%
Hemodialysis (Revenue Code: 0820-0825, 0829)	**PPR**	74.8%
Peritoneal Dialysis, CAPD and CCPD (Continuous Ambulatory Peritoneal Dialysis and Continuous Cycling Peritoneal Dialysis) (Revenue Code: 0830-0835, 0839-0845, 0849-0855, 0859)	**PPR**	74.8%
Neuropsychological Testing and Biofeedback *for NON-PSYCHIATRIC disorders only (Revenue Code: 0900, 0917-0918)	**PPR**	74.8%
Other Diagnostic Services (Revenue Codes: 0920, 0929 without CPT codes 95805-95811)	**PPR**	74.8%
Sleep Studies (Revenue Codes 0740, 0920, 0929 with CPT codes 95805-95811)	**PPR**	74.8%
Peripheral Vascular Lab (Revenue Code: 0921)	**PPR**	74.8%
EMG (Revenue Code: 0922)	**PPR**	74.8%
Allergy Testing (Revenue Code: 0924)	**PPR**	74.8%
Other Therapeutic Services (Revenue Codes: 0940, 0949)	**PPR**	74.8%
Education and Training (Revenue Code: 0942)	**PPR**	74.8%
Cardiac Rehabilitation Therapy (Revenue Code: 0943)	**PPR**	74.8%
Pulmonary Rehabilitation (Revenue Code: 0948)	**PPR**	74.8 %

(1) If the rendering of Emergency Covered Services, Urgent Care, Observation, Rule Out Myocardial Infarction or False Labor services occurs during the three calendar days immediately preceding an Admission of a Customer for the same diagnosis as defined by CMS in connection with the fee for service Medicare program, charges for such Covered Services shall not be billed by Facility separately and will not be paid by Payer or by the Customer but shall be included in the contract rate for the Admission.

2.3.2 Outpatient Procedures.

Outpatient Procedure: This section applies to Covered Services rendered to a Customer that involves a Procedure, as listed in Exhibit 1 and Exhibit 2 of this Appendix, performed in an outpatient unit of Facility ("Outpatient Procedure"). For Outpatient Procedures, the contract rate will be based on a designated group number, as set forth in the table below and as further described in this paragraph and Exhibit 1. Unless otherwise specified in this Appendix, payment under this contract rate, less any applicable Customer Expenses, shall be considered payment in full for all Covered Services rendered to Customer during an Outpatient Procedure. The group numbers below correspond with certain Outpatient Procedures identified in Exhibit 1 and Exhibit 2 to this Appendix. Facility is required to identify procedures by revenue code and CPT/HCPCS code to receive payment. United may revise the information in Exhibit 1 and Exhibit 2 based on updated Outpatient Procedure grouping information developed by CMS and on newly published codes, which may be modified by United to include procedures that are not maintained by CMS, but are considered for payment under this Appendix. The codes identified in Exhibit 1, that are

Covered Services, are considered eligible for payment under this section 2.3.2. Any updates to the information in Exhibit 1 and Exhibit 2 will be furnished to Facility upon request.

Table 3A: Outpatient Procedure Grouper

Outpatient Procedures (Revenue Codes 0360, 0361, 0369, 0481, 0490, 0499, 0750, 0790, and appropriate CPT or HCPCS Codes.) See Exhibit 1 for Revenue Code and CPT or HCPCS code criteria.	
Group Number	Per Case Contract Rate
0	$ 175.00
1	$ 450
2	$ 630
3	$ 1,500
4	$ 2,000
5	$ 2,750
6	$ 3,000
7	$ 6,000
8	$ 8,500
9	$ 10,750
10	$ 17,000
Unlisted	$ 630

2.3.3 Multiple Outpatient Procedures. When multiple Outpatient Procedures, including unlisted Outpatient Procedures, are performed on a Customer by Facility during one Outpatient Encounter, the contract rate is as follows: (1) the highest contract rate specified in section 2.3.2 for which an Outpatient Procedure has been performed; plus (2) 50% of the contract rate specified in section 2.3.2 for the Outpatient Procedure performed with the second highest contract rate. No additional payments for additional Outpatient Procedures performed during that Outpatient Encounter shall be made; instead, such additional Outpatient Procedure will be considered to have been included in the contract rate for the first two Outpatient Procedures.

2.3.4 Multiple Per Case Covered Services. If Gamma Knife, Rule Out Myocardial Infarction, False Labor, Observation, Emergency, and/or Urgent Care Covered Services are provided within a single Outpatient Encounter along with one or more Outpatient Procedures (as specified in section 2.3.2), a contract rate will apply only to the Outpatient Procedure; the Gamma Knife, Rule Out Myocardial Infarction, False Labor, Observation, Emergency, and/or Urgent Care service will be considered to have been included in the contract rate for the Outpatient Procedure.

If the Customer receives any Covered Services for which a Per Case or PPR Per Case Payment Method applies, all Covered Services during a single Outpatient Encounter that would otherwise have a contract rate pursuant to a Per Visit or Percentage Payment Rate Payment Method shall instead be included in the Per Case or PPR Per Case contract rate.

If more than one Covered Service subject to a Per Case or PPR Per Case contract rate. Payment Method applies during a single Outpatient Encounter (as specified in section 2.3), the contract rate will be the rate applicable to the Covered Service with the highest ranking, as indicated in the Case Rate Service Ranking table below. No additional payments for additional Covered Services provided during that same single Outpatient Encounter, for which a Per Case or PPR Per Case contract rate. Payment Method applies, shall be made; instead, such additional Covered Services will be considered to have been included in the contract rate for the Covered Service with the highest ranking on the table below.

Services on the Case Rate Service Ranking table below are ranked from the highest ranking to the lowest ranking, with Outpatient Procedures, as identified in Section 2.3.2, having the highest ranking.

Case Rate Service Ranking
Outpatient Procedures, as identified in Section 2.3.2
Gamma Knife
Rule Out Myocardial Infarction
False Labor
Observation
Emergency Room
Urgent Care

In the event Gamma Knife is rendered in conjunction with Outpatient Procedures, Facility may appeal for reprocessing that applies the contracted rate of Gamma Knife instead of the contract rate of Outpatient Procedure and United will reprocess the claim accordingly.

2.3.5 Outpatient Separately Reimbursable Drugs The contract rate for Separately Reimbursable Drugs (Revenue Codes 0343, 0344, and 0636) that are Covered Services will be calculated in addition to the contract rates set forth elsewhere in this Appendix, at the Per Unit via Fee Schedule # 13106 rate set forth on the UHC Separately Reimbursable Drug Fee Schedule for that Separately Reimbursable Drug Covered Service. However, if that Separately Reimbursable Drug Covered Service is rendered in conjunction with a Covered Service listed in Table 5 in Section 3.5, the contract rate for the separately reimbursable drug will apply but no additional contract rate under Section 3.5 will apply. The UHC Separately Reimbursable Drug Fee Schedule in effect on the effective date of this Appendix is attached to this Appendix as an exhibit.

Calculation of the contract rate for Separately Reimbursable Drugs are based on the following:

- Facility is required to identify procedures by revenue code and CPT/HCPCS code to receive payment.
- The fee schedule rates for existing codes will be updated quarterly. The rates will be set based on the same methodology used to establish the previous quarter's rates.
- Codes for a valid Separately Reimbursable Drug not listed on the fee schedule will have a contract rate of 50% PPR unless or until such code is added to the fee schedule as part of the quarterly update process.
- There will be a quarterly update to the fee schedule to set fixed rates for new codes published throughout the previous quarter and other Separately Reimbursable Drug codes to be added to the fee schedule. The contract rates will be set based on the same methodology used to establish the contract rates for the existing codes in the UHC Separately Reimbursable Drugs Fee Schedule.

SECTION 3
Miscellaneous Provisions

3.1 Inclusive Rates. The contract rates established by this Appendix are all-inclusive for the provision of all Covered Services to the Customer that are in the category of services that corresponds to the contract rate and which are generally provided as a part of the service. No additional payments shall be made for any services or items covered under the Customer's Benefit Plan and billed for separately by Facility.

3.2 Payment Code Updates. United shall have the right to update any codes, such as MS-DRG, Revenue Code, ICD-9-CM Code, HCPCS Code and/or CPT Code from time to time according to changes in the industry, including among other things (a) the latest edition of the Current Procedural Terminology (CPT) manual which is revised by the American Medical Association, (b) the latest edition of the HCPCS manual which is revised by the Centers for Medicare and Medicaid Services (CMS), (c) the latest edition of the ICD-9-CM which is issued by the U.S. Department of Health and Human Services, (d) the latest guidelines from the National Uniform Billing Committee and (e) the latest MS-DRG information and guidelines as published by the Centers for Medicare and Medicaid Services (CMS).

3.3 Facility-based Physician and Other Provider Charges. Facility will make its best efforts to assure that all Facility-based Physicians and other providers are participating providers as long as this Agreement is in effect.

3.4 Services Always Included Within the Contract Rate for Other Covered Services and Not Eligible for Consideration in Other Calculations for Payment. Services or items listed below and billed with indicated codes, are not subject to additional payment and are not considered in any calculation for payment, including the calculation of "lesser of" determinations under section 2.1 of this Appendix. When these services are Covered Services, per the Customer's Benefit Plan, Facility may not bill and collect from the Customer for the services, as prohibited under the Agreement. However, when these services are not Covered Services, per the Customer's Benefit Plan, Facility may bill and collect from the Customer for the services, as permitted under the Agreement.

Table 4: Services Always Included Within the Contract Rate for Other Covered Services and Not Eligible for Consideration in Other Calculations for Payment

Revenue Code	Description	Revenue Code	Description
0167	Self Care	0277	Oxygen/Take Home
0180-0189	Leave of Absence	0624	FDA Invest Device
0220-0229	Special Charges	0670-0679	Outpatient Special Residence
0253	Drugs/Take Home	0931-0932	Med Rehab Day Program
0256	Drugs/Experimental	0990-0999	Patient Convenience
0273	Supply/Take Home	2100, 2102, 2104-2109	Alternative Therapy Services

3.5 Services that may or may not be Separately Reimbursed but that are always Eligible for Consideration in Other Calculations for Payment. Services or items listed in the table below and billed with indicated codes, when they are Covered Services and rendered in conjunction with a Covered Service subject to a Per Case, Per Visit, Per Diem, MS-DRG, Per Unit, or Per Unit via Fee Schedule Payment Method under this Appendix, are always considered included in other services. These Covered Services are therefore not subject to additional payment.

Services or items billed with listed codes in the table below, when they are Covered Services, but are not rendered in conjunction with a Covered Service subject to a Per Case, Per Visit, Per Diem, MS-DRG, Per Unit, or Per Unit via Fee Schedule Payment Method under this Appendix, are subject to a Percentage Payment Rate of 74.8 % of Eligible Charges for the Covered Service, less any applicable Customer Expenses. However, this Percentage Payment Rate is subject to change under Section 4 as a result changes to Facility's Customary Charges. Additionally, adjustments to the PPR rate pursuant to Section 4 of this Appendix will carry forward into subsequent years.

Regardless of which of the above paragraphs apply, the Eligible Charges in the table below are considered in other calculations for payment, including the calculation of "lesser of" determinations under Section 2.1 of this Appendix. The Customer shall not be billed for these Covered Services. When these services are not Covered Services, per the Customer's Benefit Plan, Facility may bill and collect from the Customer for the services, as permitted under the Agreement.

Table 5: Services that may or may not be Separately Reimbursed but that are always Eligible for Consideration in Other Calculations for Payment

Revenue Code	Description	Revenue Code	Description
0230-0239	Nursing Increment	0500	OP Service
0240-0249	All Inclusive Ancillary	0509	OP/Other
0250	Pharmacy	0510-0511, 0514-0520, 0523-0526, 0529	Clinic
0251	Drugs/Generic	0530-0539	Osteopathic Services
0252	Drugs/Non Generic	0541	Ambulance/Supply
0254	Drugs/Incidental Other DX	0544	Ambulance/Oxygen
0255	Drugs/Incidental Radiology	0621	Med-Surg Sup/Incident Radiology
0257	Drugs/Nonprescription	0622	Med-Surg Supplies Incident ODX

Revenue Code	Description	Revenue Code	Description
0258	IV Solutions	0623	Surgical Dressing
0259	Drugs/Other	0631	Single Source Drug
		0632	Multiple Source Drug
0261	IV Therapy/Infusion Pump	0633	Restrictive Prescription
0262	IV Therapy/RX Svs	0634	EPO<10,000 Units
0263	IV Therapy /Drug/Supply Delv	0635	EPO 10,000 or More Units
0264	IV Therapy/Supplies	0636 *	Drugs Requiring Detailed Coding
		0637	Self Administrable Drugs Not requiring Detailed Coding
0270	Medical Surgical Supplies	0681	Trauma Level 1
0271	Non Sterile Supply	0682	Trauma Level 2
0272	Sterile Supply	0683-0689	Trauma Response
0274	Prosthetic/Orthotic Device	0700	Cast Room
0275	Pacemaker	0710	Recovery Room
0276	Intraocular Lens	0723	Circumcision
0278	Supply/Implants	0760-0761, 0769	Treatment Room
0279	Supply/Other	0770 - 0771	Preventative Care Svr/General
0343-0344 *	Nuclear Medicine - Diagnostic/Therapeutic Radiopharmaceuticals	0780	Telemedicine
0370-0379	Anesthesia	0800-0809	IP Renal Dialysis
0380-0389	Blood/Blood Products	0880-0881, 0889	Dialysis Miscellaneous
0390	Blood Administration and Processing	0946	Complex Med Equip
0391	Blood Administration	0947	Complex Med Equip/Ancillary
0392	Blood Processing and Storage	0950-0952	Other Therapeutic Services
0399	Other Blood Handling	2101	Acupuncture
		2103	Massage

*If Eligible Charges are not given an additional contract rate in accordance with Section 2.3.5 of this Appendix, then the contract rate will be determined according to this Section 3.5

3.6 Services Not Payable Under This Appendix But May Be Payable Under Another Appendix To This Agreement Or Under Another Agreement. Services or items billed with listed codes in the table below are not subject to payment under this Appendix. In the event Facility is a party to another Agreement with United or an affiliate of United that is applicable to those services rendered to a Customer, or another Appendix to this Agreement applies to these services rendered to a Customer the services below may be payable under that Appendix or Agreement.

Table 6: Services Not Payable Under This Appendix But May be Payable Under Another Appendix To This Agreement Or Under Another Agreement.

Revenue Code	Description	Revenue Code	Description
0114	Psych/Room & Board/Private	0560-0569	Medical Social Services
0116	Detox/Private	0570-0579	Home Health -Home Health Aide
0124	Psych/2bed	0580-0589	Home Health -Other Visits

Revenue Code	Description	Revenue Code	Description
0126	Detox/2bed	0590	Home Health -Units of Service
0134	Psych/3&4 bed	0600-0609	Home Health Oxygen
0136	Detox/3&4 bed	0640-0649	Home IV Therapy Services
0144	Psych/Room & Board Pvt/Deluxe	0650-0654, 0657-0659	Hospice Services
0146	Detox/Pvt/Deluxe	0660-0669	Respite Care
0154	Psych/Ward	0810-0819	Donor Bank/Bone, Organ, Skin, Bank
0156	Detox/Ward	0882	Dialysis/Home Aide Visit
0204	ICU/Psych	0901-0910	Psychiatric/Psychological Treatments
0213	CC/Transplant	0911-0916, 0919	Psychiatric/Psychological Services
0290-0299	Durable Medical Equipment	0941	Recreation/RX
0362	OR/Organ Transplant	0944	Drug Rehab
0367	OR/Kidney Transplant	0945	Alcohol Rehab
0512	Clinic – Dental Clinic	0960-0989	Professional Fees
0513	Clinic – Psychiatric Clinic	1000-1005	Behavioral Health Accommodations
0521-0522, 0524-0525, 0527-0528	Rural Health Clinic(RHC)/Federally Qualified Health Clinics(FQHC)	3101-3109	Adult Care
0550-0559	Home Health -Skilled Nursing		
MS-DRGs	**Description**	**MS-DRGs**	**Description**
001-002 w/o ICD9 37.63-37.66	Heart Transplant	009	Bone Marrow Transplant
005-006	Liver Transplant	010	Pancreas Transplant
007	Lung Transplant	652	Kidney Transplant
008	Pancreas/Kidney Transplant		

3.7 This Section Intentionally Left Blank.

3.8 Transfer of Customer. This section applies only when a Per Case, Per Case plus Per Diem after Threshold or MS-DRG Payment Method applies to all or some of the Covered Services rendered by a Facility, with regard to an Admission in which Facility makes a transfer of the Customer. A transfer is when a Customer is admitted to Facility and is subsequently transferred for additional treatment. If the length of stay of the Admission in Facility is equal to or less than the National Geometric Mean Length of Stay (GMLOS) (as published by CMS) less one, the contracted rate will be determined according to this section 3.8, rather than the contract rate that would otherwise apply under this Appendix.

If Facility receives a transfer, the contract rate is determined under this Appendix without regard to this section 3.8.

3.8.1 Transfer from Facility to a short term acute care facility or to post acute care (for those MS-DRGs designated as qualified discharges by the Centers for Medicare and Medicaid Services (CMS) except for MS-DRGs designated by CMS as "special pay" MS-DRGs). The contract rate under this section 3.8.1 is determined based on an imputed per diem rate. The imputed per diem rate is determined by dividing i) the applicable contract rate that would otherwise apply under this Appendix by ii) the GMLOS. The contract rate for the first day of the Admission is two times the imputed per diem rate and the contract rate for each subsequent day of the Admission is the imputed per diem rate; however, the contract rate under this section 3.8.1 will not exceed the contract rate that

would otherwise have applied under this Appendix.

3.8.2 Transfer from Facility to post acute care for MS-DRGs designated by CMS as "special pay" MS-DRGs. The contract rate under this section 3.8.2 is determined based on an imputed per diem rate as described above in section 3.8.1. The contract rate for the first day of the Admission is 50% of the contract rate that would otherwise have applied under this Appendix plus the imputed per diem rate. The contract rate for all subsequent days of the Admission is 50% of the imputed per diem rate; however, the contract rate under this section 3.8.2 will not exceed the contract rate that would otherwise have applied under this Appendix.

3.8.3 Temporary Transfer. If a Customer is temporarily transferred by Facility, without being discharged from Facility, for services arranged by Facility, facility services (including the services provided at the facility that receives the temporary transfer) will be paid to Facility as one continuous Admission. In such case, Facility is responsible for reimbursing the facility that receives the temporary transfer.

If a Customer is temporarily transferred to Facility, without being discharged from the transferring facility, for services arranged by the transferring facility, Facility will bill the transferring facility for those services, and neither Customer nor Payer shall be billed.

If Facility temporarily transfers Customer, without discharging Customer from Facility, via ambulance for services arranged by the Facility, charges for the ambulance services are included in the Facility's inpatient contract rate as determined in Section2.2 and neither Customer nor Payer shall be billed separately.

3.8.4 CMS Modifications. In the event that CMS modifies its approach to reimbursing for transfer in the Medicare Inpatient Prospective Payment System (IPPS), the transfer methodology under this section 3.8 will be revised consistent with the CMS changes.

3.9 Impact of Section 4 on Percentage Payment Rate or Percentage Payment Rate Per Case Rates in this Appendix. Any PPR or PPR Per Case rates in this Appendix are subject to change under Section 4 as a result of changes to Facility's Customary Charges. Additionally, adjustments to the PPR or PPR Per Case rate pursuant to Section 4 of this Appendix will carry forward into subsequent years.

SECTION 4
Adjustment to Percentage Payment Rate Resulting from Changes to Facility's Customary Charges

4.1 Intent. The intent of this section is to allow Facility to modify its Customary Charges when and how Facility chooses, while assuring that increases to Facility's Customary Charges do not have the impact of increasing the amount paid by Payers under this Appendix. For the purpose of this section, all reference to Percentage Payment Rate (PPR) encompasses both Percentage Payment Rate (PPR) and Percentage Payment Rate Per Case (PPR Per Case).

4.2 Duty to give notice. Facility shall make reasonable commercial efforts to notify United, at least 60 days prior to the implementation date, of any of the following:

(a) any increase by Facility to the Customary Charges for a significant number of Covered Services;

(b) any increase (or cumulative increases within a contract year) made by Facility to the Customary Charge for particular Covered Services that, due to the amount of the increase or to the volume of the services, is likely to have a significant cost impact on Payers. For purposes of this notice requirement, a significant cost impact is an impact of 1% or more in the total amount to be paid by Payers for commercial business under this Appendix;

(c) any increase by Facility to an algorithm or formula used by Facility to determine the mark-up to be applied by Facility, in setting its Customary Charge, to the acquisition price of products purchased by Facility and subsequently provided by Facility as a Covered Service (for instance, a drug or an implant); or

(d) any change in Facility's standard practice regarding Customary Charge changes (for instance, if

Facility's standard practice changes from making a comprehensive change in Customary Charges every January 1 to making such a change on a different date, or more than once per year).

4.3 Content of notice. The notice will include:

(a) the effective date of the chargemaster changes;

(b) Facility's estimate of the PPR rounded up or down to the first digit to the right of the decimal point, going forward, at which the cost to Payers of PPR Covered Services will be the same as it was prior to the Customary Charge increase.

The above estimates, and the review of those calculations in any subsequent informal dispute resolution or arbitration proceeding (as described below), will be calculated using the most recent available 12 months of data regarding case-mix and utilization for United's commercial book of business. The estimates will be done assuming that utilization and case-mix will be the same in the upcoming period as it was in the previous period. Neither Facility's calculations, nor the analysis in any subsequent dispute resolution proceeding of whether those calculations were correct, will attempt to measure or adjust for anticipated or actual changes in that data in the period following Facility's notice of chargemaster increase.

4.4 Cooperation with United. Facility will cooperate with United in the administration of this section by:

i) timely meeting with United, upon United's request, to discuss and explain the data provided by Facility under section 4.3 of this Appendix; this includes explaining how Facility calculated the changes to the PPR and the fixed rates (impacted by lesser of logic) identified in Facility's notice, and why Facility believes these numbers to be correct; and

ii) promptly providing, upon United's request, Facility's complete chargemaster, in the format and medium requested by United.

4.5 Adjustment to PPR and the fixed rates (impacted by lesser of logic). Upon receiving the notice from Facility described above, United will adjust the PPR and the fixed rates (impacted by lesser of logic) using the estimates provided in Facility's notice. Based on Facility's notice, United will create and implement an amended version of this Appendix; the amended Appendix will be identical to this Appendix except that it will include the revised PPR and the fixed rates (impacted by lesser of logic) set forth in Facility's notice. Provided that the amended Appendix contains no other changes, United may implement it without Facility's signature. United will provide Facility with a copy of the Amended Appendix and will indicate the effective date of the adjustments to the PPR and the fixed rates (impacted by lesser of logic).

4.6 United's right to audit. United may periodically conduct audits in connection with this section 4. The purpose of the audit may be to identify any instance in which Facility did not give the required notice, or provided inaccurate information, or provided incorrect estimates of the necessary changes to the PPR and the fixed rates (impacted by lesser of logic). Facility will cooperate with the audit process.

4.7 Recovery of overpayments. In the event that United determines that an overpayment to Facility has resulted due to Facility's failure to give timely notice as required under this section, or due to Facility providing inaccurate information, or due to Facility providing incorrect estimates of the adjustments needed to the PPR and the fixed rates (impacted by lesser of logic), United may recover those overpayments by giving notice to Facility. The notice will identify United's basis for believing that an overpayment has occurred, and further identify how United will recover the overpayment and how United will prospectively adjust the PPR and the fixed rates (impacted by lesser of logic) to prevent additional overpayments from occurring.
United will timely meet with Facility, upon Facility's request, to discuss and explain the information in United's notice, how United calculated that information, and why United believes this information to be correct.

In the event that Facility initiates dispute resolution as further described under section 4.8, the recovery and adjustments described in this section 4.7 will not take place until the conclusion of the dispute resolution process.

4.8 Dispute resolution. In the event Facility disagrees with United as to the existence of an overpayment or the amount of the overpayment, the issue will be resolved through the dispute resolution process set forth in the Agreement.

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UHC Outpatient Separately Reimbursable Drugs Fee Schedule Exhibit

Exhibit 1
Outpatient Procedure Grouper (OPG)
Revenue Code and CPT/HCPCS Code Criteria

The following is a comprehensive list of revenue codes and CPT/HCPCS codes for which the contract rate is determined according to the Outpatient Procedure Grouper Table under Section 2.3.2. If any CPT/HCPCS code from the ranges below is not listed on the OPG (Exhibit 2) then the contract rate is at the unlisted rate specified in Table 3A within section 2.3.2, less any applicable Customer Expenses. In the event a revenue code from below is billed with a CPT/HCPCS code that is not listed below, the detail line item that includes that revenue code and CPT/HCPCS code is not eligible for consideration for reimbursement. However, if that detail line item is the only detail line item that has a revenue code from the listing below, then the parties will consult as needed, at an operational level, to review the circumstances of the claim and assign appropriate CPT/HCPCS coding. Facility may resubmit the claim with the corrected coding information for consideration for reimbursement pursuant to this Appendix.

REVENUE CODE:
0360, 0361, 0369
0481
0490, 0499
0750
0790

WITH CPT / HCPCS CODES FROM THE FOLLOWING RANGES
10000 – 36299
36420 – 38200
38205 – 38206
38220 – 48548
48999 – 55899
56405 – 59320
59409 – 59409
59412 – 59414
59612 – 59612
59812 – 69990
91000 – 91299
92018 – 92019
92502 – 92502
92950 – 92998
93501 – 93662
93745 – 93745
93799 – 93799
G0104 – G0105
G0121 – G0121
G0127 – G0127
G0168 – G0168
G0173 – G0173
G0186 – G0186
G0251 – G0251
G0259 – G0260
G0267 – G0268
G0290 – G0291
G0297 – G0300
G0339 – G0340
G0364 – G0364
G0392 – G0393
0016T – 0019T
0027T – 0027T

0046T – 0056T
0061T – 0063T
0066T – 0067T
0071T – 0073T
0075T – 0084T
0088T – 0088T
0090T – 0102T
0123T – 0124T
0135T – 0137T
0155T – 0177T
0186T – 0186T